

Started on	Sunday, 12 February 2023, 8:22 PM
State	Finished
Completed on	Sunday, 12 February 2023, 8:23 PM
Time taken	11 secs
Marks	0/70
Grade	0 out of 100



Question 1

Not answered

Marked out of 1

Which of the following is NOT typically associated with school refusal?

Select one:

- ☐ Social withdrawal
- ☐ Depressed mood
- ☐ Somatic complaints
- ☐ Educational underachievement
- ☐ Onset with school transition

Your answer is incorrect.

Explanation:

School refusal is characterised by refusal to go to school or stay in school, even when under pressure from parents and school authorities. It is not a psychiatric disorder per se and can have many causes; it is considered a problem rather than a diagnosis. School refusal is generally associated with separation anxiety and may co-occur with anxiety and depressive disorders. It is usually not an isolated problem. These children typically avoid many other social situations, and often display tantrums, desperate pleas, and somatic complaints (e.g. headaches, stomach aches, nausea). School refusal may occur in a young child who is first entering school or in an older child or adolescent who is making a transition into a new grade or school.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 896.

Elliott JG, Place M. Practitioner review: school refusal: developments in conceptualisation and treatment since 2000. Journal of Child Psychology and Psychiatry. 2019 Jan;60(1):4-15.

The correct answer is: Educational underachievement

Question 2

Not answered

Marked out of 1

Which of the following childhood psychiatric conditions has the highest heritability factor?

Select one:

- ☐ Conduct disorder
- ☐ ADHD
- ☐ Separation anxiety disorder
- ☐ Oppositional defiant disorder
- ☐ School refusal

Your answer is incorrect.

Explanation:

ADHD has the highest heritability among the listed conditions. ADHD has a heritability of 70-80%, and the risk of ADHD in siblings is 2-3x increased. Rates are increased in premature babies (especially extreme prematurity), babies born to mothers who used substances during pregnancy (e.g. tobacco or alcohol), following head injury, and in some genetic and metabolic disorders. None of the proposed prenatal risk factors have been confirmed as causal for ADHD.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 668.

Sciberras E, Mulraney M, Silva D, Coghill D. Prenatal risk factors and the etiology of ADHD—review of existing evidence. Current psychiatry reports. 2017 Jan 1;19(1):1.

The correct answer is: ADHD

Question 3

Not answered

Marked out of 1

When compared to younger children, which one of the following features of depression is seen more commonly in older children and adolescents?

Select one:

- ☐ Tantrums/outbursts
- ☐ Low self-esteem
- ☐ Decreased interest in play
- ☐ Somatic complaints
- ☐ Crying

Your answer is incorrect.

Explanation:

Younger children (ages 3-8) with depression may have trouble verbalising feelings but show marked decreased interest in play, increased somatic complaints, and crying or shouting outbursts. Older children and adolescents (ages 9-18) with depression may have low self-esteem, guilt, hopelessness, increased boredom, feelings of wanting to run away, and decreased grades and poor school performance.

Ref: Mullen S. Major depressive disorder in children and adolescents. Mental Health Clinician. 2018 Nov;8(6):275-83.

The correct answer is: Low self-esteem

Question 4

Not answered

Marked out of 1

In children with PANDAS, which of the following is least common?

Select one:

- ☐ Psychosis
- ☐ Depression
- ☐ Obsessions
- ☐ Anxiety
- ☐ Tics

Your answer is incorrect.

Explanation:

Paediatric autoimmune neuropsychiatric disorder associated with streptococcal infections (PANDAS) is characterised by the sudden and dramatic onset of obsessive-compulsive disorder, tic symptoms, or severely restricted food intake and neuropsychiatric comorbidity, also of sudden and severe onset, following group A streptococci (GAS) infection. Common comorbidities include anxiety (including separation anxiety); emotional lability and/or depression; irritability, aggression and/or severely oppositional behaviours; behavioural (developmental) regression; deterioration in school performance (ADHD-like symptoms, memory deficits, cognitive changes); sensory or motor abnormalities; somatic signs and symptoms (including sleep disturbances, enuresis, or increased urinary frequency). Nearly 1/3 of children endorse concerns related to suicide. The term Pediatric Acute-onset Neuropsychiatric Syndrome (PANS) is used when the etiology or mechanism of the abrupt onset is unknown.

Ref: Warrilow A, Morton M. Autoimmune disorders in child psychiatry: keeping up with the field. BJPsych Advances. 2015 Nov;21(6):367-76.

Cabrera-Mendoza B et al. Diagnosis in PANDAS: An Update. Current Psychiatry Research and Reviews Formerly: Current Psychiatry Reviews. 2019 Dec 1;15(4):237-47.

The correct answer is: Psychosis

Question 5

Not answered

Marked out of 1

All of the following statements about the Wechsler Intelligence Scale for Children are true except

Select one:

- ☐ Object assembly and coding are two subtests used to assess performance IQ
- ☐ It can be given to children from 6 to 17 years of age
- ☐ It is helpful for the assessment of general intelligence in school-age children
- ☐ Tests are divided into 'verbal' and 'performance' categories, yielding a verbal and performance IQ
- ☐ Twelve primary subtests are usually used, measuring different aspects of the child's ability

Your answer is incorrect.

Explanation:

Wechsler Intelligence Scale for Children (third edition):

- Most widely used test of intelligence for school-age children and adolescents
- Can be given to children from 6 to 17 years of age
- Yields a verbal IQ, performance IQ, and combined full-scale IQ
- Verbal subtests (5): vocabulary, information, arithmetic, similarities, comprehension (and one supplemental: digit span)
- Performance subtests (5): block design, picture completion, picture arrangement, object assembly, coding (and two supplemental: mazes and symbol search)
- The IQ computation does not include the scores of the supplemental subtests

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 81.

The correct answer is: Twelve primary subtests are usually used, measuring different aspects of the child's ability

Question 6

Not answered

Marked out of 1

About 90% of children are dry at night by the age of

Select one:

- ☐ 8 years
- ☐ 4 years
- ☐ 5 years
- ☐ 3 years
- ☐ 2 years

Your answer is incorrect.

Voluntary control does not begin until 15-18 months. Most children are reasonably dry by day at 18 months. By 2, years 50% are dry at night, by 3 years 75% are dry at night and by 5 years 90% are dry at night.

The correct answer is: 5 years



Question 7

Not answered

Marked out of 1

Which one of the following is a feature seen in truants rather than school refusers?

Select one:

- ☐ Antisocial symptoms
- ☐ Family history of neurosis
- ☐ Satisfactory academic achievement
- ☐ Over-protective parenting
- ☐ Small family

Your answer is incorrect.

Truancy is phenomenologically different from School Refusal. School Refusers are characterised by the presence of emotional rather than antisocial symptoms, with a familial tendency towards neurosis rather than antisocial behaviour, with parents being over-protective rather than inconsistent. The academic achievement in school refusers is usually satisfactory whereas truants have poor track record. Small family size is associated with school refusal while larger families have more truants.

The correct answer is: Antisocial symptoms



Question 8

Not answered

Marked out of 1

Which of the following is true about Tourette syndrome?

Select one:

- ☐ Mean age of onset is 12 years
- ☐ Females are more affected than males
- ☐ Diagnosed by having both motor and vocal tics for 6 months
- ☐ Diagnosed by having both motor and vocal tics for 12 months
- ☐ Antipsychotics are the first line treatment

Your answer is incorrect.

Explanation:

Tourette syndrome is a complex neuropsychiatric disorder with childhood onset characterised by multiple motor and one or more vocal tics present for at least a year. Males are more commonly affected (3-4:1, male:female). The age of onset is 2-21 years, with a mean age of 7 years. All patients should receive psychoeducation, reassurance, and explanation. In mild cases and young people this may suffice. Behavioural methods (that may be useful alone or in combination with medications) include habit reversal training, comprehensive behavioural intervention for tics, exposure and response prevention, and self-hypnosis. Medication is warranted if tics cause distress or are functionally disabling and is also used in the treatment of many of the co-morbidities.

Ref: Robertson MM. Tourette syndrome in children and adolescents: aetiology, presentation and treatment. BJPsych Advances. 2016 May;22(3):165-75.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 587.

The correct answer is: Diagnosed by having both motor and vocal tics for 12 months

Question 9

Not answered

Marked out of 1

Blockade of which of the following receptors can reduce vocal and motor tics seen in Tourette syndrome?

Select one:

- ☐ D1 receptors
- ☐ 5HT3 receptors
- ☐ 5HT1D receptors
- ☐ Dopamine reuptake transporter
- ☐ D2 receptors

Your answer is incorrect.

Explanation:

Evidence suggests that dopamine super-sensitivity, specifically involving nigrostriatal dopamine D2 receptors, leads to tics. Agents that reduce dopamine activity suppress tics, while dopamine-enhancing substances (like cocaine) exacerbate them.

Ref: Kaufman DM, Geyer HL, Milstein MJ, Rosengard JL. Kaufman's Clinical Neurology for Psychiatrists, 9th Edition. 2023. p. 444.

Nomura Y. Pharmacological therapy for Tourette syndrome: What medicine can do and cannot do. Biomedical Journal. 2021 Sep 20.

The correct answer is: D2 receptors

Question 10

Not answered

Marked out of 1

Which of the following is thought to have the least genetic loading/heritability?

Select one:

- ☐ Nocturnal enuresis
- ☐ Tourette syndrome
- ☐ Bipolar disorder
- ☐ Conduct disorder
- ☐ Childhood-onset schizophrenia

Your answer is incorrect.

Explanation:

Of those listed, conduct disorder is thought to have the least genetic loading. Still, there is now solid evidence from twin and adoption studies that conduct problems assessed both dimensionally and categorically are substantially heritable.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the recognition, intervention and management of antisocial behaviour and conduct disorders in children and young people. Updated 2017.

The correct answer is: Conduct disorder

Question 11

Not answered

Marked out of 1

Which one of the following is a true statement about bipolar disorder in children and adolescents?

Select one:

- ☐ 60-70% of initial presentations are a manic episode
- ☐ Males predominate in adolescent cases
- ☐ Manic children typically present with atypical or mixed features
- ☐ Lamotrigine is used if treatment resistant manic episodes are present
- ☐ The course is often episodic and of brief duration

Your answer is incorrect.

Explanation:

Children experiencing a manic episode typically present with atypical or mixed features characterised by irritability, labile mood, and behavioural problems. Periods of depression are common, and early-onset bipolar disorder more often presents with depression than in adult-onset bipolar. Early-onset bipolar may be characterised by non-episodic, chronic, ultra-rapid cycling, mixed irritable and manic states. The prevalence in adolescents is approximately 1% and as high as 6% when including soft subsyndromal symptoms. M>F in childhood cases; M=F in adolescents. For the treatment of mania and hypomania in youth, NICE guidelines suggest following the same recommendations as adults: second-generation antipsychotics (SGAs) may be used as first-line treatment, and mood stabilisers can be added after failure of two trials of SGAs. Lamotrigine has not been studied in RCTs for the treatment of acute bipolar depression in children and adolescents and is not recommended as first-line. It is not recommended in the treatment of manic episodes.

Ref: Findling RL et al. Progress in diagnosis and treatment of bipolar disorder among children and adolescents: an international perspective. Evidence-based mental health. 2018 Nov 1;21(4):177-81.

National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 547-551.

The correct answer is: Manic children typically present with atypical or mixed features



Question 12

Not answered

Marked out of 1

Which of the following risk factors is associated with autism spectrum disorder?

Select one:

- ☐ Exposure to MMR vaccine
- ☐ Premature birth
- ☐ Overcrowded family
- ☐ Having very strict parents
- ☐ Low socioeconomic status of parents

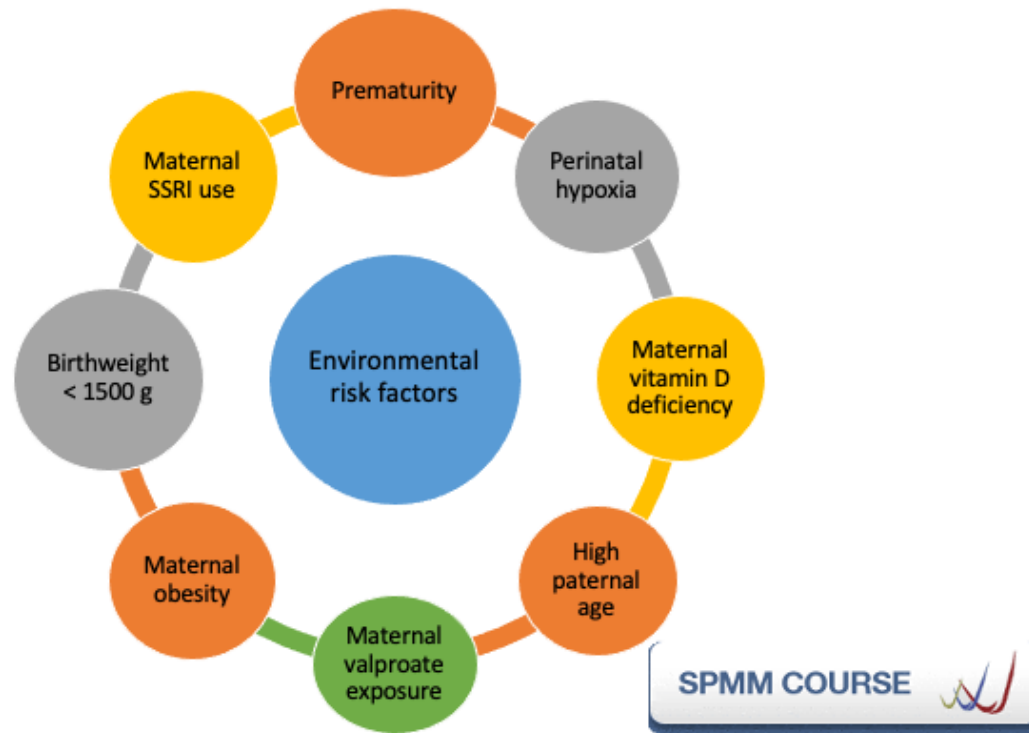
Your answer is incorrect.

Explanation:

The prevalence of autism spectrum disorder in preterm infants is considerably higher than the general population.



Environmental risk factors for autism spectrum disorder



Ref: Agrawal S, Rao SC, Bulsara MK, Patole SK. Prevalence of autism spectrum disorder in preterm infants: a meta-analysis. *Pediatrics*. 2018 Sep 1;142(3):e20180134.

The correct answer is: Premature birth

Question 13

Not answered

Marked out of 1

Which of the following treatments is most recommended for treating PTSD in children?

Select one:

- ☐ Trauma-focused CBT
- ☐ Treatment with SSRIs
- ☐ Psychological debriefing
- ☐ Relaxation and social skills training
- ☐ Psycho-education

Your answer is incorrect.

Explanation:

Within one month of a traumatic event, consider active monitoring or individual trauma-focused CBT for children and adolescents. >1 month after a traumatic event, the treatment of choice for PTSD is individual trauma-focused CBT. Consider EMDR for those ages 7-17 who present >3 months after a traumatic event only if they do not respond to or engage with trauma-focused CBT. Do NOT use psychologically-focused debriefing for the prevention or treatment of PTSD in children and adolescents. Do NOT offer drug treatments for the prevention or treatment of PTSD in children and young people under 18 years.

Ref: National Institute for Health and Clinical Excellence. NICE guideline Post-traumatic stress disorder 2018.

The correct answer is: Trauma-focused CBT

Question **14**

Not answered

Marked out of 1

The prevalence of ADHD in the UK is

Select one:

- ☐ 2-5%
- ☐ 0.5-1%
- ☐ 1 -2%
- ☐ 10-15%
- ☐ 5-10%

Your answer is incorrect.

Explanation:

The UK prevalence for ADHD is just under 5%; 3-4% in boys and just under 1% in girls (four times more common in boys).

Ref: Hill P. Attention-deficit hyperactivity disorder in children and adolescents: Assessment and treatment. BJPsych Advances. 2015 Jan;21(1):23-30.

The correct answer is: 2-5%



Question 15

Not answered

Marked out of 1

The sex distribution is equal in which of the following conditions?

Select one:

- ☐ School refusal
- ☐ Tic disorders
- ☐ Conduct disorder
- ☐ ADHD
- ☐ Truancy

Your answer is incorrect.

School refusal is characterised by refusal to go to school or stay in school, even when under pressure from parents and school authorities. It is not a psychiatric disorder per se and can have many causes; it is considered a problem rather than a diagnosis. The incidence in children and adolescents reported to be 1-5%, with the sex distribution equal.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 896.

Elliott JG, Place M. Practitioner review: school refusal: developments in conceptualisation and treatment since 2000. Journal of Child Psychology and Psychiatry. 2019 Jan;60(1):4-15.

The correct answer is: School refusal

Question 16

Not answered

Marked out of 1

In a 7-year-old child with uncomplicated Tourette syndrome, the drug treatment of choice is

Select one:

- ☐ Atomoxetine
- ☐ Clonidine
- ☐ Sertraline
- ☐ Haloperidol
- ☐ Methylphenidate

Your answer is incorrect.

Explanation:

Tourette syndrome is a complex neuropsychiatric disorder with childhood onset characterised by multiple motor and one or more vocal tics present for at least a year. All patients should receive psychoeducation, reassurance, and explanation. In mild cases and young people this may suffice. Behavioural methods (that may be useful alone or in combination with medications) include habit reversal training, comprehensive behavioural intervention for tics, exposure and response prevention, and self-hypnosis. Medication is warranted if tics cause distress or are functionally disabling and is also used in the treatment of many of the co-morbidities. Adverse effects of antipsychotics may outweigh beneficial effects in the treatment of tics and so it is recommended that clonidine or guanfacine are always tried first. Antipsychotics may, however, be more effective than adrenergic alpha-2 agonists in alleviating tics in some individuals.

Ref: Robertson MM. Tourette syndrome in children and adolescents: aetiology, presentation and treatment. BJPsych Advances. 2016 May;22(3):165-75.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 587-588.

The correct answer is: Clonidine

Question 17

Not answered

Marked out of 1

What percentage of patients with autism have intellectual disability?

Select one:

- ☐ 70-80%
- ☐ 10-15%
- ☐ 5%
- ☐ 40-50%
- ☐ 20-40%

Your answer is incorrect.

Explanation:

80% of patients with autism have mild to moderate intellectual disability. In general terms, 1-2% of those with autism have a 'normal' life; 5-20% have a 'borderline' prognosis (i.e. varying degrees independence), but 70% are totally dependent upon support.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 822.

The correct answer is: 70-80%



Question **18**

Not answered

Marked out of 1

The ratio of prevalence of ADHD in males to females is

Select one:

- ☐ 3 to 1
- ☐ 2 to 1
- ☐ 4 to 1
- ☐ 1 to 1
- ☐ 1 to 2

Your answer is incorrect.

Explanation:

The UK prevalence for ADHD is just under 5%; 3-4% in boys and just under 1% in girls (four times more common in boys).

Ref: Hill P. Attention-deficit hyperactivity disorder in children and adolescents: Assessment and treatment. BJPsych Advances. 2015 Jan;21(1):23-30.

The correct answer is: 4 to 1



Question 19

Not answered

Marked out of 1

Which of the following is not a characteristic feature of PTSD in children?

Select one:

- ☐ Avoidance
- ☐ Hyperarousal
- ☐ Intrusive re-experiencing of a traumatic event
- ☐ Nightmares
- ☐ Selective amnesia for the event

Your answer is incorrect.

Explanation:

The clinical presentation of PTSD in children may include the following:

- Compulsive repetitive play representing part of the trauma and failing to relieve anxiety
- Recurrent, involuntary, and intrusive distressing memories of the traumatic event(s)
- Intrusive re-experiencing of the event(s)
- Dissociative reactions (e.g. flashbacks) in which the child feels or acts as if the traumatic event(s) were recurring
- Avoidance
- Nightmares, sleep terrors, and/or difficulty going to sleep
- Constriction of play
- Social withdrawal
- Restricted affect

- Loss of acquired developmental skills, especially language regression and toilet training
- Hyperarousal, hypervigilance, exaggerated startle response
- Decreased concentration and attention
- New separation anxiety
- New aggression

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 688.

American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.

The correct answer is: Selective amnesia for the event



Question 20

Not answered

Marked out of 1

A child is hyperactive and distractible in school and at home. Choose the risk factor associated with the development of this condition.

Select one:

- ☐ Maternal benzodiazepine use in pregnancy
- ☐ Measles vaccination
- ☐ Maternal SSRI use in pregnancy
- ☐ Maternal smoking in pregnancy
- ☐ Maternal cannabis use in pregnancy

Your answer is incorrect.

Explanation:

ADHD has a heritability of 70-80%, and the risk of ADHD in siblings is 2-3x increased. Rates are increased in premature babies (especially extreme prematurity), babies born to mothers who used substances during pregnancy (e.g. tobacco or alcohol), following head injury, and in some genetic and metabolic disorders. None of the proposed prenatal risk factors have been confirmed as causal for ADHD.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 668.

Sciberras E, Mulraney M, Silva D, Coghill D. Prenatal risk factors and the etiology of ADHD—review of existing evidence. Current psychiatry reports. 2017 Jan 1;19(1):1.

The correct answer is: Maternal smoking in pregnancy

Question **21**

Not answered

Marked out of 1

A 14-year-old boy was interviewed to identify the presence of psychiatric disease. What scale could be used again at the age of 20?

Select one:

- ☐ Clinical Interview Schedule-Revised
- ☐ Schedule for Assessment in Neuropsychiatry
- ☐ Present State Examination
- ☐ Structured Clinical Interview
- ☐ Children's Depression Inventory

Your answer is incorrect.

Explanation:

The Clinical Interview Schedule-Revised (CIS-R) asks questions about a range of symptoms and can be used to assign ICD diagnoses of psychiatric disorders.

Ref: Cornish RP, John A, Boyd A, Tilling K, Macleod J. Defining adolescent common mental disorders using electronic primary care data: a comparison with outcomes measured using the CIS-R. BMJ open. 2016 Dec 1;6(12).

The correct answer is: Clinical Interview Schedule-Revised

Question **22**

Not answered

Marked out of 1

You want to follow the progress in symptoms for a period of 3 years in patients diagnosed with depression at the age of 14. Which clinical instrument is most useful?

Select one:

- ☐ Hamilton Depression Rating Scale
- ☐ Cornell Depression Scale
- ☐ SCID (Structured Clinical Interview for DSM)
- ☐ Beck Depression Inventory
- ☐ Child and Adolescent Functional Assessment Scale

Your answer is incorrect.

Explanation:

The Child and Adolescent Functional Assessment Scale (CAFAS) is a clinician-rated measure of functional impairment in youth 6-17 years of age, secondary to behavioural, emotional, or substance use problems.

Ref: Boydell KM, Barwick M, Ferguson HB, Haines R. A feasibility study to assess service providers' perspectives regarding the use of the Child and Adolescent Functional Assessment Scale in Ontario. The Journal of Behavioral Health Services & Research. 2005 Jan 1;32(1):105-9.

The correct answer is: Child and Adolescent Functional Assessment Scale

Question **23**

Not answered

Marked out of 1

Which of the following blood abnormalities is commonly seen in anorexia nervosa?

Select one:

- ☐ Raised MCV
- ☐ Eosinophilia
- ☐ Macrocytosis
- ☐ Reduced MCH
- ☐ Normocytic normochromic anaemia

Your answer is incorrect.

Explanation:

Anaemia, leukopenia, and although less frequently, thrombocytopaenia are possible haematological complications of anorexia nervosa (AN) considered strictly secondary to chronic malnutrition. Usually, anaemia in AN is normocytic and normochromic, and leukopenia is characterised by lymphocytopenia or neutropenia; thrombocytopaenia may also be present.

Ref: De Filippo E et al. Hematological complications in anorexia nervosa. European Journal of Clinical Nutrition. 2016 Nov;70(11):1305-8.

The correct answer is: Normocytic normochromic anaemia

Question **24**

Not answered

Marked out of 1

What is the average age of onset of schizophrenia?

Select one:

- ☐ 35
- ☐ 30
- ☐ 15
- ☐ 25
- ☐ 20

Your answer is incorrect.

Explanation:

The average age of onset of schizophrenia is 23 years of age in men and 26 years of age in women.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 190.

The correct answer is: 25



Question 25

Not answered

Marked out of 1

Which one of the following statements about suicidal behaviour in adolescents is true?

Select one:

- ☐ 1% of adolescents who attempt suicide repeat within a year
- ☐ Self-poisoning is the commonest form of parasuicide
- ☐ Suicide is the leading cause of death for adolescents
- ☐ Female suicides outnumber male suicides at all ages
- ☐ Most adolescents who make suicide attempts actually have a psychiatric disorder

Your answer is incorrect.

Suicide is the third leading cause of death for adolescents, following accidents and homicides (Hawton 1986). Few adolescents who make suicide attempts actually have a psychiatric disorder and serious suicidal intent is generally low but if present, can be serious. Self-poisoning is the commonest form of parasuicide, the usual agents being analgesics and psychotropic agents. Others would include hanging, jumping from heights, car exhaust fumes and shooting being most frequent.

The correct answer is: Self-poisoning is the commonest form of parasuicide



Question **26**

Not answered

Marked out of 1

The prevalence of suicidal ideations in the past year for adolescent cohort in Britain is

Select one:

- ☐ 15%
- ☐ 3%
- ☐ 10%
- ☐ 5%
- ☐ 25%

Your answer is incorrect.

Suicidal ideation (without deliberate self-harm) in the past year was reported by 15.0% of an adolescent cohort in UK (school pupils - self-report). This was more common in females (22%) than males (8.5%) (Odds ratio 3.1).

The correct answer is: 15%



Question **27**

Not answered

Marked out of 1

The most common psychiatric emergency seen in the child and adolescent population is

Select one:

- ☐ Homicidal behaviour
- ☐ Suicidal behaviour
- ☐ Assault and violence
- ☐ Sexually inappropriate behaviour
- ☐ Psychosis

Your answer is incorrect.

Explanation:

Numerous reasons are given for emergency presentations, but most commonly the goal is the assessment of self-injurious thoughts and behaviours (SITB). In addition to non-suicidal self-injury (NSSI), which is carried out without the intention to die, suicidal behaviour, in which the person has at least to some degree the intention to die, is defined as SITB. Since the intention to die is often ambivalent in patients, there is overlap between NSSI and suicidal behaviour.

Ref: Kandsperger S et al. Suicidality presented to a child and adolescent psychiatry emergency service: increasing rate and changing characteristics. *Frontiers in psychiatry*. 2021;1195.

The correct answer is: Suicidal behaviour

Question **28**

Not answered

Marked out of 1

Preschoolers and school-age children who stutter exhibit an increased incidence of which of the following conditions?

Select one:

- ☐ Intellectual disability
- ☐ Psychosis
- ☐ OCD
- ☐ Depression
- ☐ Social anxiety

Your answer is incorrect.

Explanation:

Very young children who stutter typically show some delay in the development of language and articulation without additional disorders of speech and language. Preschoolers and school-age children who stutter exhibit an increased incidence of social anxiety, school refusal, and other anxiety symptoms. Older children who stutter also do not necessarily have co-morbid speech and language disorders, but often manifest anxiety symptoms and disorders. When stuttering persists into adolescence, social isolation occurs at higher rates than in the general adolescent population.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 118.

The correct answer is: Social anxiety

Question **29**

Not answered

Marked out of 1

The overall prevalence of psychiatric problems in adolescence is

Select one:

- ☐ 10-15%
- ☐ 1-5%
- ☐ 16-20%
- ☐ 3-8%
- ☐ 50-60%

Your answer is incorrect.

Explanation:

Adolescence, defined as a transitional phase between ages 10 and 19, is sometimes perceived as a phase of life with no health problems. However, approximately 20% of adolescents experience a mental health problem, most commonly depression or anxiety.

Ref: Silva SA et al. Common mental disorders prevalence in adolescents: A systematic review and meta-analyses. PloS one. 2020 Apr 23;15(4):e0232007.

The correct answer is: 16-20%



Question **30**

Not answered

Marked out of 1

A child with ADHD and congenital heart disease. What is the medication of choice?

Select one:

- ☐ Clonidine
- ☐ Dexamphetamine
- ☐ Methylphenidate
- ☐ Atomoxetine
- ☐ Amitriptyline

Your answer is incorrect.

Stimulant medications should be avoided in patients with congenital structural heart diseases due to possible risk of sudden unexplained death. The Food and Drug Administration in 2006 (US) asked the manufacturers of stimulants, to add a warning to their label saying the medications should not be used in children with structural heart abnormalities or other heart problems.

The correct answer is: Atomoxetine

Question **31**

Not answered

Marked out of 1

Which autoimmune disorder is associated with OCD in children?

Select one:

- ☐ Systemic lupus erythematosus
- ☐ Wegener granulomatosis
- ☐ PANDAS
- ☐ Rheumatoid arthritis
- ☐ Gluten sensitive enteropathy

Your answer is incorrect.

Explanation:

Paediatric autoimmune neuropsychiatric disorder associated with streptococcal infections (PANDAS) is characterised by the sudden and dramatic onset of obsessive-compulsive disorder (OCD), tic symptoms, or severely restricted food intake and neuropsychiatric comorbidity, also of sudden and severe onset, following group A beta-haemolytic streptococcal (GABHS) infection. The pathophysiological hypothesis in PANDAS states that GABHS infections trigger the production of antibodies that, in turn, cross-react against the central nervous system and cause tics and OCD symptoms. In contrast to Tourette syndrome, where a preceding infection may exacerbate symptoms, in PANDAS the streptococcal infection and its induced cross-reacting antibodies are proposed as the cause of the symptoms. The term Pediatric Acute-onset Neuropsychiatric Syndrome (PANS) is used when the aetiology or mechanism of the abrupt onset is unknown.

Ref: Warrilow A, Morton M. Autoimmune disorders in child psychiatry: keeping up with the field. BJPsych Advances. 2015 Nov;21(6):367-76.

Cabrera-Mendoza B et al. Diagnosis in PANDAS: An Update. Current Psychiatry Research and Reviews Formerly: Current Psychiatry Reviews. 2019 Dec 1;15(4):237-47.

Gilbert DL et al. A pediatric neurology perspective on pediatric autoimmune neuropsychiatric disorder associated with streptococcal infection and pediatric acute-onset neuropsychiatric syndrome. The Journal of pediatrics. 2018 Aug 1;199:243-51.

The correct answer is: PANDAS

Question 32

Not answered

Marked out of 1

Sibling rivalry disorders usually emerge in which of the following time periods?

Select one:

- ☐ Within 3 months after the birth of the sibling
- ☐ Within 4 months after the birth of the sibling
- ☐ Within a year after the birth of the sibling
- ☐ Within 1 month after the birth of the sibling
- ☐ Within 6 months after the birth of the sibling

Your answer is incorrect.

Sibling rivalry disorder is characterised by a child having an abnormally intense negative feelings toward an immediately younger sibling. Emotional disturbance is shown by regression, tantrums, and dysphoria; sleep difficulties, oppositional behaviour, or attention-seeking behaviour with one or both parents (two or more of these must be present). The onset is within 6 months of the birth of an immediately younger sibling.

The correct answer is: Within 6 months after the birth of the sibling

Question 33

Not answered

Marked out of 1

In operant conditioning with children, shaping refers to

Select one:

- ☐ Reinforcing positive behaviour; ignoring negative behaviour
- ☐ Punishing negative behaviour; praising positive behaviour
- ☐ Neither reinforcing nor punishing observed behaviours
- ☐ Gradual reinforcement of behaviours approximating the desired outcome
- ☐ Differential reinforcement of behaviours other than the desired outcome

Your answer is incorrect.

Explanation:

Shaping is used to produce a complex behaviour that is not in the child's initial repertoire. Initially, component parts of the desired behaviour are rewarded, then reward is limited to behaviour that approximates the desired outcome. As appropriate behaviour appears, it is rewarded only if it is 'in the right direction', and further reward is contingent upon continued advancement until the child is only rewarded once the entire behaviour is performed.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 907.

The correct answer is: Gradual reinforcement of behaviours approximating the desired outcome

Question **34**

Not answered

Marked out of 1

Which of the following drugs has the best evidence for use in childhood depression?

Select one:

- ☐ Citalopram
- ☐ Escitalopram
- ☐ Amitriptyline
- ☐ Fluoxetine
- ☐ Paroxetine

Your answer is incorrect.

Explanation:

UK guidelines (particularly NICE, updated 2019) recommend a stepped-care approach in the treatment of childhood depression. 5–11-year-old children should only be offered fluoxetine when psychological therapy has been ineffective. Second-line medications are sertraline and citalopram. Paroxetine and venlafaxine should not be used, as they have the strongest association with suicide risk (especially with adolescents). TCAs should not be used; they are not useful in young children and there is only low-quality marginal evidence to support their use in adolescents.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the identification and management of depression in children and young people. 2019.

The correct answer is: Fluoxetine

Question 35

Not answered

Marked out of 1

Most children with selective mutism also meet diagnostic criteria for which of the following disorders?

Select one:

- ☐ Autism
- ☐ Generalised anxiety disorder
- ☐ Asperger syndrome
- ☐ Depression
- ☐ Social anxiety disorder

Your answer is incorrect.

Explanation:

Selective mutism is characterised by the persistent failure to speak in specific settings (e.g. school) where there is an expectation to speak. It begins after normal speech has been acquired, and there is full use of language at home/with family. Patients may remain wholly silent or near-silent, in some cases only whispering in a school setting. In rare cases, a child is mute at home but not in school. Social anxiety is almost always present.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 687.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 201.

The correct answer is: Social anxiety disorder

Question 36

Not answered

Marked out of 1

Which one of the following statements about Rett syndrome is true?

Select one:

- ☐ Occurs mainly in boys
- ☐ Characterised by progressive loss of acquired abilities
- ☐ Associated with borderline IQ
- ☐ Apparent lack of normal development in the first 2 years of life
- ☐ Associated with microcephaly at birth

Your answer is incorrect.

Explanation:

Rett Syndrome:

- Believed to be caused by dominant X-linked gene
- Degenerative disorder that almost exclusively affects females
- Initially normal development
- Ages 1-4 yrs: purposeful hand movements & spoken language lost; stereotypical hand movements (hand-wringing) & autism
- Ataxia, facial grimacing, teeth-grinding, and loss of speech
- Irregular respiration with episodes of hyperventilation, apnea, and breath-holding
- Progressive gait disturbance, scoliosis, and seizures
- Sleep disturbances are common; initial and/or intermediate insomnia, leading to excessive daytime sleep

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 103.

Persico AM et al. The psychopharmacology of autism spectrum disorder and Rett syndrome. In Handbook of clinical neurology 2019 Jan 1 (Vol. 165, pp. 391-414). Elsevier.

The correct answer is: Characterised by progressive loss of acquired abilities



Question **37**

Not answered

Marked out of 1

Which of the following disorders is characterised by impaired ability to recognise words, slow and inaccurate reading, and poor comprehension?

Select one:

- ☐ Developmental language disorder with impairment of mainly pragmatic language
- ☐ Developmental language disorder with impairment of receptive and expressive language
- ☐ Developmental language disorder with impairment of mainly expressive language
- ☐ Developmental learning disorder with impairment in reading
- ☐ Developmental learning disorder with impairment in written expression

Your answer is incorrect.

Explanation:

Developmental learning disorder with impairment in reading is characterised by significant and persistent difficulties in learning academic skills related to reading, such as word reading accuracy, reading fluency, and reading comprehension. The individual's performance in reading is markedly below what would be expected for chronological age and level of intellectual functioning and results in significant impairment in functioning. 4% of school-age children are affected. There is male predominance [4:1] and often a family history. Approximately 20% of children have co-morbid CD or ADHD.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 678.

The correct answer is: Developmental learning disorder with impairment in reading

Question 38

Not answered

Marked out of 1

Reactive attachment disorder is generally diagnosed before the age of

Select one:

- ☐ 10
- ☐ 2
- ☐ 5
- ☐ 3
- ☐ 12

Your answer is incorrect.

Explanation:

Reactive attachment disorder (RAD) is characterised by grossly abnormal attachment behaviours in early childhood, occurring in the context of a history of grossly inadequate child care (e.g. severe neglect, maltreatment, institutional deprivation). Even when an adequate primary caregiver is newly available, the child does not turn to the primary caregiver for comfort, support, and nurture; rarely displays security-seeking behaviours towards any adult; and does not respond when comfort is offered. RAD can only be diagnosed in children, and features of the disorder develop within the first 5 years of life. However, the disorder cannot be diagnosed before the age of 1 (or a developmental age of less than 9 months), when the capacity for selective attachments may not be fully developed, or in the context of autism spectrum disorder.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: 5

Question 39

Not answered

Marked out of 1

The most common psychiatric co-morbidity associated with Asperger syndrome is

Select one:

- ☐ Schizophrenia
- ☐ Alcoholism
- ☐ Depression
- ☐ OCD
- ☐ Bipolar affective disorder

Your answer is incorrect.

Explanation:

Psychiatric comorbidity is high in **Asperger** syndrome. **Depression** and **anxiety** are the most common. Bipolar affective disorder and schizophrenia are more common than in the general population.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 820.

The correct answer is: Depression

Question 40

Not answered

Marked out of 1

Which one of the following is a good prognostic factor in children with conduct disorder?

Select one:

- ☐ Male gender
- ☐ Onset before 10 years of age
- ☐ Having been discharged from school
- ☐ Resilient temperament
- ☐ Having a family member convicted for violent conduct

Your answer is incorrect.

Protective factors conferring a better prognosis include 1. Female gender 2. High IQ 3. Resilient temperament 4. Good parenting among several others. Werner's (1992, 1994) longitudinal study investigated resilience in over 200 babies born in 1955 on a Hawaiian island, following and assessing the children at various times up to the age of 32 years. Werner found that the resilient children - those with the ability to cope with the internal stresses of their vulnerabilities and the external stresses of their environment - had low rates of variously defined conduct problems.

The correct answer is: Resilient temperament

Question **41**

Not answered

Marked out of 1

Regarding the aetiology of ADHD, all of the following are true except

Select one:

- ☐ There is a 50% increased risk in monozygotic twins
- ☐ Siblings of hyperactive children have about twice the increased risk of having the disorder
- ☐ Predominant serotonin dysregulation is seen in the prefrontal cortex
- ☐ Neuroimaging shows lower cerebral blood flow and metabolic rates in the frontal lobe areas of children with ADHD than in controls
- ☐ Genes 5, 6, and 11 are implicated

Your answer is incorrect.

Explanation:

Many neurotransmitters are likely associated with ADHD symptoms. However, dopamine is a primary focus of clinical investigation, and the prefrontal cortex has been implicated based on its role in attention and regulation of impulse control. Studies have also shown that brain regions such as the locus coeruleus, which consists predominantly of noradrenergic neurons, play a significant role in attention. In part, hypotheses regarding the neurochemistry of ADHD have arisen from the effect of medications. Stimulants, known to be the most effective medications in the treatment of ADHD, affect both dopamine and norepinephrine, leading to neurotransmitter hypotheses that may include dysfunction in both the adrenergic and dopaminergic systems.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 139.

The correct answer is: Predominant serotonin dysregulation is seen in the prefrontal cortex

Question **42**

Not answered

Marked out of 1

Which of the following suggests a diagnosis of bulimia nervosa rather than anorexia nervosa?

Select one:

- ☐ Age of onset at 15
- ☐ Lanugo hairs
- ☐ Calorie counting
- ☐ Laxative abuse
- ☐ Poor impulse control

Your answer is incorrect.

Explanation:

There is an increased prevalence of disorders of impulsivity and lack of control in people with bulimia nervosa. Patients who have to satisfy their binges at all costs (e.g. shoplifting) may have issues with impulsivity and rage. People with bulimic illnesses also have a higher prevalence of substance misuse.

Ref: Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.

The correct answer is: Poor impulse control

Question 43

Not answered

Marked out of 1

Pica typically occurs during

Select one:

- ☐ 3-6 months of age
- ☐ 6-12 months of age
- ☐ 2-3 years of age
- ☐ 5-6 years of age
- ☐ 8-10 years of age

Your answer is incorrect.

Explanation:

Pica is characterised by the persistent (>1 month) eating of non-nutritive substances at a developmentally inappropriate age (>1 year). It typically occurs during the 2nd and 3rd years of life, though may occur in young pregnant women. Common substances include dirt, stones, plastic, paper, hair, faeces, wood, string, etc. Pica is common in people with intellectual disability and autism spectrum disorder. Consequences may include toxicity, infection, or GIT ulceration/obstruction. Hypothesized causes include hunger, malnutrition, nutritional deficiencies, psychosocial stressors, and brain disorders such as lesions in the hypothalamus.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 694.

The correct answer is: 2-3 years of age

Question **44**

Not answered

Marked out of 1

A 14-year-old girl presents with some uncoordinated movements of her limbs and emotional lability. She has a history of fever and sore throat six months ago. Which is the most likely cause for her presentation?

Select one:

- ☐ Tourette syndrome
- ☐ Sydenham chorea
- ☐ OCD
- ☐ ADHD
- ☐ Huntington chorea

Your answer is incorrect.

Explanation:

Sydenham chorea is the major neurological presentation of rheumatic fever, an inflammatory process following infection with group A streptococci (GAS) such as throat infection or scarlet fever. Sydenham chorea is characterised by uncoordinated jerking movements affecting the face and limbs, as well as hypotonia. Some children also experience vocal and motor tics that may persist. Behavioural symptoms and psychiatric co-morbidities are common and can have an abrupt onset that may precede the onset of the chorea. Emotional lability, anxiety, and symptoms of OCD are the most well recognised and there have also been associations with ADHD, mood disorders, and even symptoms of psychosis.

Ref: Warrilow A, Morton M. Autoimmune disorders in child psychiatry: keeping up with the field. BJPsych Advances. 2015 Nov;21(6):367-76.

The correct answer is: Sydenham chorea

Question **45**

Not answered

Marked out of 1

Which of the following treatments is no longer licensed in the UK for children with ADHD?

Select one:

- ☐ Clonidine
- ☐ Pemoline
- ☐ Imipramine
- ☐ Methylphenidate
- ☐ Atomoxetine

Your answer is incorrect.

Explanation:

Pemoline is no longer licensed in UK because of the risk of serious hepatotoxicity and death.

Ref: Safer DJ, Zito JM, Gardner JF. Pemoline hepatotoxicity and postmarketing surveillance. Journal of the American Academy of Child & Adolescent Psychiatry. 2001 Jun 1;40(6):622-9.

The correct answer is: Pemoline



Question 46

Not answered

Marked out of 1

Which drug is now specifically contraindicated in depressed children and adolescents due to an association with increased risk of suicide?

Select one:

- ☐ Paroxetine
- ☐ Fluoxetine
- ☐ Escitalopram
- ☐ Citalopram
- ☐ Mirtazapine

Your answer is incorrect.

Explanation:

Paroxetine and venlafaxine should not be used for the treatment of depression in children and young people, as they have the strongest association with suicide risk.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the identification and management of depression in children and young people. 2019.

The correct answer is: Paroxetine

Question **47**

Not answered

Marked out of 1

The most common cause of co-morbidity in children and adolescents with bipolar disorder is

Select one:

- ☐ Substance use disorder
- ☐ Borderline personality disorder
- ☐ Tourette syndrome
- ☐ ADHD
- ☐ Anxiety disorder

Your answer is incorrect.

Explanation:

Amongst children and adolescents with bipolar disorder, approximately 53% have ADHD, 42% ODD, 23% an anxiety disorder, 27% conduct disorder, and 9% a substance disorder.

Ref: Findling RL et al. Progress in diagnosis and treatment of bipolar disorder among children and adolescents: an international perspective. Evidence-based mental health. 2018 Nov 1;21(4):177-81.

The correct answer is: ADHD

Question 48

Not answered

Marked out of 1

Manifestations of oppositional defiant disorder are almost invariably present

Select one:

- ☐ At home
- ☐ At school
- ☐ In the presence of peers
- ☐ In the presence of all adults
- ☐ In the presence of teachers

Your answer is incorrect.

Explanation:

In oppositional defiant disorder (ODD), a child's temper outbursts, active refusal to comply with rules, and annoying behaviours exceed expectations for these behaviours for children of the same age. The disorder is an enduring pattern of negativistic, hostile, and defiant behaviours in the absence of significant violations of the rights of others. Manifestations of ODD are almost **invariably present in the home**, but they may not be present at school or with other adults or peers. In some cases, the child has symptoms outside the home from the start. In others, it starts at home and later moves beyond that. Typically, symptoms of the disorder are most evident in interactions with adults or peers whom the child knows well.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 187.

The correct answer is: At home

Question 49

Not answered

Marked out of 1

The most common co-morbid disorders seen in Tourette syndrome are

Select one:

- ☐ Conduct disorders
- ☐ Anxiety disorders
- ☐ Intellectual disabilities
- ☐ Impulse control disorders
- ☐ ADHD and OCD

Your answer is incorrect.

Explanation:

90% of patients with Tourette syndrome have at least one co-morbidity. Up to 55% of patients are also diagnosed with ADHD, 50% with OCD and up to 20% with co-morbid autism spectrum disorder. Males are more likely to have co-morbid disorders than females.

Ref: Robertson MM. Tourette syndrome in children and adolescents: aetiology, presentation and treatment. BJPsych Advances. 2016 May;22(3):165-75.

Yang Z et al. Investigating shared genetic basis across tourette syndrome and comorbid neurodevelopmental disorders along the impulsivity-compulsivity spectrum. Biological psychiatry. 2021 Sep 1;90(5):317-27.

The correct answer is: ADHD and OCD

Question **50**

Not answered

Marked out of 1

The most common co-morbidity in patients with Tourette's syndrome is;

Select one:

- ☐ ADHD
- ☐ Oppositional defiant disorder
- ☐ Conduct disorder
- ☐ Autism
- ☐ OCD

Your answer is incorrect.

Obsessive-compulsive symptoms or disorder (OCD) has been found to co-exist in 1/3rd to 2/3rds of children and adolescents with Tourette's disorder and about 1/3rd of adults with Tourette's disorder have persistent OCD into adulthood. (Kaplan and Sadock -Synopsis- Pg 1235)

The correct answer is: OCD



Question 51

Not answered

Marked out of 1

Which of the following has been found to be effective in treating adolescent inhalant abuse?

Select one:

- ☐ Olanzapine
- ☐ Naltrexone
- ☐ Nicotinamide
- ☐ Acamprosate
- ☐ No known pharmacotherapy

Your answer is incorrect.

Explanation:

Adolescent inhalant abuse is characterised by the use of gases, glues, and/or aerosols to achieve intoxication. Inhalants are frequently one of the first drugs to be used by adolescents due to ease of access, lack of penalties association with possession, and rapid onset of effects. First-line treatment for adolescent inhalant abuse is multimodal, including individual CBT, family therapy, and activity and engagement programs.

Ref: Nguyen J, O'Brien C, Schapp S. Adolescent inhalant use prevention, assessment, and treatment: A literature synthesis. International Journal of Drug Policy. 2016 May 1;31:15-24.

The correct answer is: No known pharmacotherapy

Question 52

Not answered

Marked out of 1

Which of the following should be used as a first choice treatment in adolescents with acute manic or mixed episodes?

Select one:

- ☐ Olanzapine
- ☐ Lithium
- ☐ Carbamazepine
- ☐ Sodium valproate
- ☐ Lamotrigine

Your answer is incorrect.

Explanation:

Aripiprazole and lithium are the only licensed medications for treating mania. In practice, lithium is not generally used acutely in the treatment of manic/mixed episodes in adolescents, as it requires commitment to longer-term treatment and regular blood tests. NICE guidelines (updated 2020) recommend considering aripiprazole or using an alternative antipsychotic (e.g. risperidone, olanzapine, quetiapine) in accordance with guidance for adults and limiting drug treatment to 12 weeks. It is unclear whether valproate has the same beneficial effects on mania in young people as it does in adults. NICE recommends that valproate monotherapy should NOT be used for first-line management of mania and it should NOT be offered to girls or young women of childbearing potential because of teratogenic effects and the risk of polycystic ovary syndrome.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

Findling RL et al. Progress in diagnosis and treatment of bipolar disorder among children and adolescents: an international perspective. Evidence-based mental health. 2018 Nov 1;21(4):177-81.

The correct answer is: Olanzapine

Question 53

Not answered

Marked out of 1

Which neurotransmitter system is commonly postulated to be involved in the aetiology of Tourette syndrome?

Select one:

- ☐ Glutamine
- ☐ Acetylcholine
- ☐ Dopamine
- ☐ Serotonin
- ☐ GABA

Your answer is incorrect.

Explanation:

The aetiology of Tourette syndrome includes genetic factors and environmental influences (infections and neuroimmunological effects, pre- and/or perinatal difficulties, psychosocial stressors, and androgen influences). No single gene has been identified (likely more than one gene). An association with psychosocial stress is well known. Heightened HPA axis and noradrenergic system reactivity have been demonstrated. There is a likely disturbance of the dopamine system. Other aetiological possibilities include gestational and perinatal insults, exposure to androgens, heat, fatigue, and post-infectious autoimmune mechanisms.

Ref: Robertson MM. Tourette syndrome in children and adolescents: aetiology, presentation and treatment. BJPsych Advances. 2016 May;22(3):165-75.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 676.

The correct answer is: Dopamine

Question **54**

Not answered

Marked out of 1

Which one of the following has confirmed the most evidence for successful treatment of OCD compared to any of the other anxiety disorders of childhood?

Select one:

- ☐ CBT
- ☐ Treatment with SSRIs
- ☐ Combination of both medication and CBT
- ☐ Psycho education
- ☐ Behaviour Therapy

Your answer is incorrect.

Most treatment guidelines for children and adolescents with mild to moderate OCD recommend a trial of cognitive-behavioural therapy prior to initiating medication, though there is evidence that optimal treatment includes a combination of SSRI medication and CBT.

The correct answer is: Combination of both medication and CBT



Question **55**

Not answered

Marked out of 1

The total sleeping time of a newborn baby is approximately

Select one:

- ☐ >20 hours per day
- ☐ 8-10 hours per day
- ☐ 14-17 hours per day
- ☐ 18-20 hours per day
- ☐ 11-13 hours per day

Your answer is incorrect.

Most newborns (0-3 months) sleep 14-17 hours per day.

Ref: Bathory E, Tomopoulos S. Sleep regulation, physiology and development, sleep duration and patterns, and sleep hygiene in infants, toddlers, and preschool-age children. Current problems in pediatric and adolescent health care. 2017 Feb 1;47(2):29-42.

The correct answer is: 14-17 hours per day



Question **56**

Not answered

Marked out of 1

Which is the first line drug in an 8-year-old with uncomplicated ADHD?

Select one:

- ☐ Dexamphetamine
- ☐ Methylphenidate
- ☐ Atomoxetine
- ☐ Clonidine
- ☐ Imipramine

Your answer is incorrect.

Explanation:

Methylphenidate (stimulant) is recommended as first-line medication (either short- or long-acting preparation) in children and young people with ADHD by NICE guidelines (updated 2018). 

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the diagnosis and management of attention deficit hyperactivity disorder. 2018.

The correct answer is: Methylphenidate



Question **57**

Not answered

Marked out of 1

Which of the following may be a long-term effect of prescribing high doses of methylphenidate over long periods without drug holidays?

Select one:

- ☐ Seizures
- ☐ Insomnia
- ☐ Growth slowing
- ☐ Nephrotoxicity
- ☐ Psychosis

Your answer is incorrect.

Explanation:

Stimulant medication, including methylphenidate, commonly suppresses appetite during the day. This is occasionally associated with slowing of weight growth and, less commonly, with slowing of height growth. NICE recommends that if a child or young person's height over time is significantly affected by medication (i.e. they have not met height expected for age), a planned break in treatment over school holidays should be considered.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the diagnosis and management of attention deficit hyperactivity disorder. 2018.

Hill P. Attention-deficit hyperactivity disorder in children and adolescents: Assessment and treatment. BJPsych Advances. 2015 Jan;21(1):23-30.

The correct answer is: Growth slowing

Question **58**

Not answered

Marked out of 1

The commonest age of presentation for school refusal is

Select one:

- ☐ 5 years
- ☐ 8 years
- ☐ 11 years
- ☐ 3 years
- ☐ 14 years

Your answer is incorrect.

Explanation:

School refusal is characterised by refusal to go to school or stay in school, even when under pressure from parents and school authorities. It is not a psychiatric disorder per se and can have many causes; it is considered a problem rather than a diagnosis. School refusal may occur in a young child who is first entering school or in an older child or adolescent who is making a transition into a new grade or school. The peak age is 11-13 years. The incidence in children and adolescents reported to be 1-5%, with the sex distribution equal.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 896.

Elliott JG, Place M. Practitioner review: school refusal: developments in conceptualisation and treatment since 2000. Journal of Child Psychology and Psychiatry. 2019 Jan;60(1):4-15.

The correct answer is: 11 years

Question 59

Not answered

Marked out of 1

Which of the following disorders is more common in females?

Select one:

- ☐ Developmental language disorder with impairment of receptive and expressive language
- ☐ Developmental learning disorder with impairment in written expression
- ☐ Developmental learning disorder with impairment in mathematics
- ☐ Developmental learning disorder with impairment in reading
- ☐ Developmental language disorder with impairment of mainly expressive language

Your answer is incorrect.

Explanation:

Developmental learning disorder with impairment in mathematics is characterised by significant and persistent difficulties in learning academic skills related to mathematics or arithmetic, such as number sense, memorisation of number facts, accurate calculation, fluent calculation, and accurate mathematic reasoning. The individual's performance in mathematics or arithmetic is markedly below what would be expected for chronological or developmental age and level of intellectual functioning and results in significant impairment in functioning. 1-6% of school-age children are affected, with female predominance. It is often associated with visuospatial deficits and attributed to right parietal dysfunction.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 678.

The correct answer is: Developmental learning disorder with impairment in mathematics

Question 60

Not answered

Marked out of 1

All of the following are risk factors for adolescent suicide except

Select one:

- ☐ Alcohol abuse
- ☐ Depression
- ☐ Past history of attempted suicide
- ☐ Female sex
- ☐ Family history of suicide

Your answer is incorrect.

Explanation:

Rates of suicide in the UK have increased among adolescents by approximately 7.9% each year between 2010 and 2017. Rates among females have increased at a higher rate, though absolute number of deaths is still lower among young women than young men. Completed suicide is more common in males, while suicide attempts and self-harm are more common in females. Other risk factors for adolescent suicide include depression, struggles with sexuality or gender, aggression and impulsivity, difficulties in peer relationships, family difficulties, sleep difficulties, history of self-harm and/or attempted suicide, parental psychopathology & suicidal behaviour, and substance misuse (including alcohol).

Ref: Correia R, Jackson D. Risk to self: identifying and managing risk of suicide and self-harm. BJPsych Advances. 2020;1-11.

Bould H et al. Rising suicide rates among adolescents in England and Wales. The Lancet. 2019 Jul 13;394(10193):116-7.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 698-699.

The correct answer is: Female sex

Question **61**

Not answered

Marked out of 1

Which of the following is associated with conduct disorder?

Select one:

- ☐ Increase in nicotinic receptor count
- ☐ Low levels of adenosine
- ☐ Low CSF serotonin
- ☐ Low blood brain barrier permeability
- ☐ High levels of dopamine

Your answer is incorrect.

Explanation:

Studies of conduct-disordered juvenile offenders have found high plasma serotonin levels in the blood. Evidence indicates that blood serotonin levels correlate inversely with levels of 5-HIAA in the CSF and that low 5-HIAA levels in the CSF correlate with aggression and violence.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 194.

The correct answer is: Low CSF serotonin

Question 62

Not answered

Marked out of 1

Which of the following symptoms of mania is least commonly reported in children with bipolar disorder?

Select one:

- ☐ Distractibility
- ☐ Increased energy
- ☐ Hypersexuality
- ☐ Irritability
- ☐ Elated mood

Your answer is incorrect.

Explanation:

Symptoms of mania commonly reported in children with bipolar disorder include increased energy, irritability, mood lability, distractibility, goal-directed activity, euphoria/elated mood, pressured speech, hyperactivity, racing thoughts, poor judgement, grandiosity, inappropriate laughter decreased need for sleep, and flight of ideas.

Ref: Findling RL et al. Progress in diagnosis and treatment of bipolar disorder among children and adolescents: an international perspective. Evidence-based mental health. 2018 Nov 1;21(4):177-81.

The correct answer is: Hypersexuality

Question 63

Not answered

Marked out of 1

Developmentally inappropriate social relatedness seen before the age of 5 following prolonged separation from caregivers is seen in

Select one:

- ☐ Disinhibited social engagement disorder
- ☐ Oppositional defiant disorder
- ☐ Conduct disorder
- ☐ Separation anxiety disorder
- ☐ Childhood disintegrative disorder

Your answer is incorrect.

Explanation:

Disinhibited social engagement disorder is newly described in both DSM-5 and ICD-11. It is characterised by a pattern of behaviour in which a child actively approaches and interacts with unfamiliar adults in an overly familiar way. These children may display reduced/absent reticence in approaching/interacting with unfamiliar adults; overly familiar verbal or physical behaviour; diminished/absent checking back with an adult caregiver; and willingness to go off with unfamiliar adults with little to no hesitation. These children have experienced patterns of extremes of insufficient care (e.g. social neglect or deprivation, repeated changes of primary caregivers, rearing in institutions or other settings that severely limit opportunities to form selective attachment).

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Disinhibited social engagement disorder

Question **64**

Not answered

Marked out of 1

The prevalence of ADHD among girls in the UK between the ages of 5-15 is

Select one:

- ☐ 2-3%
- ☐ 8-10%
- ☐ 12-15%
- ☐ 0-1%
- ☐ 5-6%

Your answer is incorrect.

Explanation:

The UK prevalence for ADHD is just under 5%; 3-4% in boys and just under 1% in girls (four times more common in boys).

Ref: Hill P. Attention-deficit hyperactivity disorder in children and adolescents: Assessment and treatment. BJPsych Advances. 2015 Jan;21(1):23-30.

The correct answer is: 0-1%



Question **65**

Not answered

Marked out of 1

The overall prevalence of psychiatric disorders in adolescence is

Select one:

- ☐ 6-10%
- ☐ 16-20%
- ☐ 2-5%
- ☐ 11-15%
- ☐ 21-25%

Your answer is incorrect.

Explanation:

Adolescence, defined as a transitional phase between ages 10 and 19, is sometimes perceived as a phase of life with no health problems. However, approximately 20% of adolescents experience a mental health problem, most commonly depression or anxiety.

Ref: Silva SA et al. Common mental disorders prevalence in adolescents: A systematic review and meta-analyses. PloS one. 2020 Apr 23;15(4):e0232007.

The correct answer is: 16-20%



Question **66**

Not answered

Marked out of 1

What proportion of adolescents admits to having had suicidal ideas in the UK in the past year?

Select one:

- ☐ 25%
- ☐ 1%
- ☐ 10%
- ☐ 15%
- ☐ 5%

Your answer is incorrect.

In the UK an estimated 15% of adolescents admit to suicidal ideation in the past year.

The correct answer is: 15%



Question **67**

Not answered

Marked out of 1

The percentage of reading difficulties in children in Britain is estimated to be around

Select one:

- ☐ 1%
- ☐ 0.05%
- ☐ 0.2%
- ☐ 25%
- ☐ 5%

Your answer is incorrect.

In the UK (and in most of the English-speaking developed world) it is generally recognized that between 4 to 7% of children have specific reading disorder - developmental dyslexia - at any given time.

The correct answer is: 5%



Question 68

Not answered

Marked out of 1

Tom is a 7-year-old boy who was recently diagnosed with ADHD. His mum wants to know about the long term prognosis. Which of the following symptoms is most likely to remit first?

Select one:

- ☐ Impulsivity
- ☐ Hyperactivity
- ☐ Aggressiveness
- ☐ Distractibility
- ☐ Inattention

Your answer is incorrect.

Explanation:

A proportion of children with ADHD will improve with maturation. Although hyperactivity wanes with age, impulsivity and inattention do so at a slower rate. About 15% of children who show a full ADHD picture in late childhood will have a full diagnosis in early adult life. 50% will show an attenuated picture characterized by troublesome inattentiveness, impulsiveness, and personal disorganisation.

Ref: Hill P. Attention-deficit hyperactivity disorder in children and adolescents: Assessment and treatment. BJPsych Advances. 2015 Jan;21(1):23-30.

The correct answer is: Hyperactivity

Question 69

Not answered

Marked out of 1

According to the Isle of Wight survey, the commonest psychiatric disorder in childhood is

Select one:

- ☐ Conduct disorder
- ☐ Anxiety disorders
- ☐ Depression
- ☐ Substance abuse
- ☐ ADHD

Your answer is incorrect.

In the Isle of White study, CD was found to be the most common psychiatric disorder amongst 10-11 year olds.

The correct answer is: Conduct disorder



Question 70

Not answered

Marked out of 1

The Cambridge study by Farrington et al. identified risk factors in children between 8-10 years that can predict later delinquent behaviour by the age of 32. Which one among the following is not a risk factor identified by this study?

Select one:

- ☐ Family criminality
- ☐ Antisocial behaviour
- ☐ ADHD
- ☐ Low intelligence
- ☐ Parental depression

Your answer is incorrect.

Explanation:

The most important predictors, at age 8-10, of later delinquency from the Cambridge study:

- Antisocial child behaviour, including troublesomeness in school, dishonesty, and aggressiveness
- Hyperactivity-impulsivity-attention deficit
- Low intelligence and poor school attainment
- Family criminality
- Family poverty
- Poor parental child-rearing behaviour

Ref: Farrington DP. The Twelfth Jack Tizard Memorial Lecture* The development of offending and antisocial behaviour from childhood: Key findings from the Cambridge study in delinquent development. Journal of Child Psychology and Psychiatry. 1995 Sep;36(6):929-64.

The correct answer is: Parental depression



© 2023 SPMM Course Limited, UK.

[Refund Policy](#) | [Terms & Conditions](#)

